TPF induction: docetaxel, cisplatin and fluorouracil

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction Days 2–4 · fluorouracil continues

Active medicines: Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Continuation/delayed antiemetic prescription	Prescriber/pharmacist to complete As prescribed As prescribed	Days 2–4 fluorouracil and two days after systemic anticancer therapy (SACT), accounting for delayed Day 1 cisplatin risk Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Retain prescribed delayed cisplatin antiemetics; no new docetaxel/cisplatin infusion.
- Fluorouracil: continuous intravenous (IV) infusion over 96 hours on Days 1–4; the source also permits daily 22-hour infusions on Days 1–4. Record the actual selected option, pump/bag changes and disconnect time.
- Document dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) phenotype and/or genotype before first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting-dose plan. Renal impairment can elevate uracil and misclassify phenotype; consider genotype. A normal result does not exclude all severe toxicity.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00315 v7a · pNCCP 00315: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00315 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required